MOOD-03 | Therapist only - all age groups

Choosing an action and adapting it to capacity

1 | Purpose, fit and boundaries

Purpose of this unit

Choose a small personal action when daily life has become more limited, adapt it to available energy and resources, and prepare an alternative for a difficult day. The outcome is a flexible attempt, not a commitment to feel better.

Choice and adaptation

Choose this sheet following professional assessment and with consent. The person may skip questions, answer aloud, or choose another tool. This is proposed original material for review; it provides no score, diagnosis, or promise of an outcome.

Boundaries specific to this unit

Do not set a uniform activity or effort quota, interpret difficulty as unwillingness, or recommend a change in treatment. Assess acute distress, declining functioning, or medical limitations before using the sheet. Do not force activity or social contact.

Reusing existing material

MOOD-01 and MOOD-02 provide a map of the day and its barriers. Core-05 already offers a small step; here it is adapted to low mood, demands, and an alternative plan. Refer to an existing map without completing it again.

> Draft for administrator review. Clinical review and final rights approval are still pending.

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Working together with the worksheet

2 | A conversation about one situation

Opening

Choose a situation the person wants to explore. It need not be the hardest situation. Check that the question is understood and the wording fits the person; their account comes before the therapist's interpretation.

1 | Question for discussion

What important area of my life has become more limited as my mood has fallen?

2 | Question for discussion

What action could I take without needing to feel good first?

3 | Question for discussion

How can I adapt its size to my energy and commitments?

4 | Question for discussion

What conditions or support would make a manageable attempt possible?

5 | Question for discussion

What alternative will I choose if the plan does not fit that day?

Closing

Summarize what became clearer and what remains open. Use the existing SES-S01 and SES-S02 for the session framework. There is no need to repeat an assessment or plan already recorded.

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Adapting for four age groups

3 | Age is a starting point, not an assumption about ability

CH | Children, approximately ages 6 to 11

Offer a choice between two concrete actions, not a task list. Agree on adult help with the child, and do not measure success by compliance.

Offer one question at a time with little writing. Adult support must not replace the child's own voice.

TE | Adolescents, approximately ages 12 to 17

Respect school and relationship demands. Do not turn the attempt into a required report to a parent or teacher.

Agree in advance what will be shared and with whom, and explain the privacy limits in this setting. Do not promise absolute confidentiality.

AD | Adults

Explore commitments, living conditions, and resources. A possible action may also be asking for help or reducing demands.

OA | Older adults

Ask about preferences, accessibility, and fatigue without assuming limitations or a need for a companion. Do not increase physical activity without adapting it to the person's circumstances.

Offer reading support, time, and conversation according to preference. Choose help from another person together with the participant, respecting their privacy.

> Each age version needs its own review. Approval of one version does not automatically approve the others.

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Practice, review and next steps

4 | Shared choice and learning from the attempt

Before offering practice sheet P

Decide together whether practice between sessions is appropriate at all. Define one opportunity, boundaries, and desired support. Practice may take place only in the session. There is no required daily sequence or requirement to share the sheet with others.

At the next session

Review fit and learning, not just completion. Revisit barriers if the step was not possible; do not double tasks to make up a debt.

Professional and technical records

Each version has a separate review record covering age suitability, originality, rights, and fictional filling. Reviewer and date fields remain blank until assigned. File checks or a code merge are not clinical approval. Clinical information stays only in the authorized setting, outside the code repository.

Background references

The following links were used for background only. Questions and adaptations were written afresh for the expansion plan; no questionnaire, scale, or protocol was copied. Adult sources do not validate the child versions. This material makes no claim about session count or treatment effectiveness.

www.nice.org.uk/guidance/ng222/chapter/Recommendations
www.nice.org.uk/guidance/ng134/chapter/Recommendations

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